

Content and Clinical Governance Document

Swasthya Katha — Safe, Reviewed, Multilingual Health Content

Version: 1.0

Status: Draft for clinical and product approval

Date: 24 August 2026

1. Purpose

This document defines how health content is researched, written, illustrated, translated, narrated, reviewed, published, updated, and retired.

The goal is not merely to produce attractive content. The goal is to make sure every public lesson is:

- Medically responsible.
- Understandable.
- Non-stigmatizing.
- Appropriate for the intended audience.
- Correct in every supported language.
- Accessible to people with disabilities.
- Traceable to a source.
- Reversible when guidance changes.

2. Governing principles

1. No medical claim is published without an identified source.
2. No clinical claim is approved by AI alone.
3. Visuals must not imply diagnosis from appearance.
4. Translations require human native-language review.
5. A visual change can trigger clinical review if its meaning changes.
6. Narration, captions, text, and visuals must remain semantically aligned.
7. Local referral instructions require local ownership and review.
8. Content expiry and re-review dates are mandatory.
9. Uncertainty must be stated plainly rather than hidden.
10. Users must be directed to qualified services for personal concerns.

3. Scope of first publication

The first publication may address STI and sexual-health education. Proposed modules:

- What an STI is.
- How transmission can occur.
- The fact that some infections may have no visible signs.
- Prevention and risk reduction.
- Testing and what to expect.
- Treatment and professional care.
- Communication and stigma reduction.
- Myths and facts.
- Local help and referrals.

The SlideShare reference can inform topic discovery, but it must not be treated as an automatically approved source, and external images must not be reused without rights, clinical review, and audience-suitability review. [web:10]

4. Source hierarchy

Use sources in this order:

1. Applicable national clinical guidance.
2. Ministry or department of health material.
3. WHO guidance and fact sheets.
4. Recognized public-health agencies.
5. Peer-reviewed guidance.
6. Expert-reviewed local educational resources.

WHO material is a useful authoritative foundation for STI scope, prevention, diagnosis, and treatment, but local clinical guidance and referral information must be checked before release. [web:27]

5. Claim register

Every claim receives a unique record:

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Claim ID:
Original claim:
Plain-language version:
Medical topic:
Population:
Country/region:
Source title:
Source URL or citation:
Source publication date:
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Evidence type:
Clinical reviewer:
Approval status:
Review date:
Next review date:
Translation dependencies:
Visual dependencies:

6. Content lifecycle

Research

- > Outline
- > Claim register
- > Draft
- > Plain-language adaptation
- > Clinical review
- > Storyboard
- > Illustration
- > Accessibility description
- > Translation
- > Native-language review
- > Narration
- > Timing and visual synchronization
- > Accessibility review
- > Product approval
- > Versioned publication
- > Monitoring
- > Re-review or retirement

7. Roles and responsibilities

Content author

- Drafts the lesson.
- Maintains the claim register.
- Avoids unsupported claims.
- Responds to review comments.

Clinical reviewer

- Verifies medical accuracy.
- Checks population and country scope.
- Checks risk of misinterpretation.
- Approves, rejects, or requests revision.

Public-health reviewer

- Checks practical usefulness.
- Verifies prevention and referral framing.
- Checks alignment with program goals.

Visual designer

- Converts approved meaning into a visual system.
- Avoids decorative ambiguity.
- Provides target IDs and fallback descriptions.

Translator

- Produces a natural local-language adaptation.
- Flags concepts that do not transfer directly.
- Does not silently change medical meaning.

Native-language reviewer

- Checks terminology, tone, clarity, and cultural appropriateness.

Narrator/audio producer

- Records approved scripts.
- Maintains pronunciation and tone.
- Provides clean audio and transcript alignment.

Accessibility reviewer

- Reviews descriptions, text alternatives, focus order, captions, contrast, and motion.

Product owner

- Confirms scope, risk acceptance, and release readiness.

8. Plain-language standard

Use:

- One idea per screen.
- Short sentences.
- Familiar words.
- Direct active voice.
- Concrete examples.

- Repetition of the safe action.
- Explanations of medical terms.

Avoid:

- Blame.
- Shame.
- Jargon without explanation.
- Absolute guarantees.
- Diagnosis by appearance.
- Complex conditional sentences.
- Unnecessary statistics.
- Scary or sensational imagery.

9. Clinical safety review

The reviewer must answer:

- Is each medical claim correct?
- Is the claim current?
- Does the claim apply to this population and country?
- Could the wording be interpreted as a diagnosis?
- Could the reader delay necessary care?
- Does the advice accidentally imply a guarantee?
- Is urgent-care guidance required?
- Is treatment wording too specific for a general educational product?
- Is the referral information accurate?
- Does the illustration communicate the same meaning?

10. Visual content policy

Preferred

- Abstract diagrams.
- Human figures.
- Clinic journeys.
- Action icons.
- Colour-coded but labelled toolkits.
- Anatomical diagrams only when clinically justified.

Restricted

- Graphic photographs.
- Images that can be interpreted as diagnostic examples.
- Images without license or consent.
- Humiliating or stigmatizing portrayals.
- Images copied from search results.

11. AI controls

AI may assist with:

- Outline proposals.
- Plain-language alternatives.
- Translation suggestions.
- Ambiguity detection.
- Code and validation.

AI may not:

- Approve medical claims.
- Invent sources.
- Publish content.
- Decide the clinical meaning of an image without review.
- Replace native-language review.
- Generate individualized treatment instructions.

12. Translation and narration governance

Translation begins only after source text is clinically approved. The translated package includes:

- Text.
- Visual labels.
- Accessibility description.
- Narration script.
- Captions.
- Quiz wording.
- Action and referral messages.

Each locale must pass a separate review. Timing data is locale-specific because word length and sentence structure change.

13. Publication states

Draft
Content review
Clinical review
Translation in progress
Language review
Accessibility review
Product review
Approved for release
Published
Scheduled for review
Deprecated
Withdrawn

14. Publication gate

A lesson cannot publish unless:

- Claims are sourced.
- Clinical review is approved.
- Translation is approved for the advertised locale.
- Audio and transcript match.
- Visual alternatives exist.
- Assets pass licensing and sanitization checks.
- Accessibility review passes.
- Version and review date are recorded.

15. Re-review policy

Review content when:

- A source guideline changes.
- A treatment or testing recommendation changes.
- A referral service changes.
- A translation changes meaning.
- A visual changes clinical interpretation.
- A safety complaint is received.
- The scheduled review date arrives.

Suggested target: begin routine review at least 30 days before expiry.

16. Incident process

If unsafe content is found:

1. Disable or withdraw the lesson.
2. Notify the clinical owner.
3. Preserve the affected version.
4. Assess user exposure.
5. Correct the claim and visual.
6. Obtain review.
7. Publish the corrected version.
8. Record the root cause and prevention.

17. Evidence required

Store:

- Source register.
- Review decisions.
- Annotated changes.
- Final scripts.
- Translation approvals.
- Audio QA.
- Visual descriptions.
- License records.
- Publication manifest.
- Re-review schedule.

18. Acceptance criteria

This document is approved when:

- Clinical owner is named.
- Source hierarchy is approved.
- Claim register format is accepted.
- Review roles are assigned.
- AI restrictions are accepted.
- Image policy is accepted.
- Publication gate is testable.
- Incident process has an owner.

